

ELECTRONIC MEDICAL RECORD

Comprehensive Patient Chart Export
New Amandaville Regional Health System

Patient Name:	Gardner, Leslie	Patient ID:	59034db9-be2f-4977...
Date of Birth:	1979-09-07	Sex:	F
Address:	529 Kristi Ridge Apt. 581, New Amandaville, OR 97112	Payer:	Kaiser
Member ID:	KAIS-62C8B9B303	MRN:	MRN-8827301

Record Summary

This record contains an abbreviated chart export for the patient identified above, spanning three (3) distinct clinical encounters related to the evaluation, surgical treatment, and resolution of a chronic left foot/ankle peripheral nerve compression (tarsal tunnel syndrome secondary to a ganglion cyst). Encounters are presented in chronological order and include the initial orthopedic consultation, the inpatient surgical encounter, and the final follow-up visit confirming symptom resolution.

Encounter Index

#	Date of Service	Setting	Specialty	Summary
1	2024-06-15	Outpatient	Orthopedic Surgery	Initial consultation for left toe numbness/tingling
2	2024-12-21	Inpatient / Surgical	Orthopedic Surgery	Left foot exploration & ganglion cyst excision
3	2025-03-15	Outpatient	Neurology	Final follow-up; symptom resolution

Immunization History

Vaccine	Date Administered	Notes
Influenza (annual)	2024-10-02	Left deltoid, no adverse reaction
Tdap	2021-03-11	Left deltoid, no adverse reaction
COVID-19 (bivalent booster)	2023-11-08	Right deltoid, no adverse reaction

Consents / Advance Directives on File

General consent to treat and financial responsibility agreement on file (signed at registration). HIPAA Notice of Privacy Practices acknowledgment on file. Informed consent for surgical procedure (left foot exploration and ganglion cyst excision) signed and on file for the 2024-12-21 surgical encounter. No advance directive or healthcare power of attorney on file as of the most recent encounter; the patient was offered information regarding advance care planning and declined to complete documentation at this time.

Active Problem List (as of most recent encounter)

Problem	Managing Service
Resolved/resolving left tarsal tunnel neuropathy, s/p surgical decompression	Neurology / Ortho
Essential hypertension, controlled	Primary Care
Mild intermittent asthma, controlled	Primary Care
Primary hypothyroidism, controlled	Primary Care
History of anxiety	Primary Care
History of mild anemia	Primary Care

Insurance & Authorization Summary

The patient is covered under a Kaiser health plan, Member ID KAIS-62C8B9B303, active throughout the period covered by this record. Prior authorization was obtained for the MRI of the left foot/ankle performed on 2024-11-14 and for the inpatient surgical encounter on 2024-12-21. Referrals between primary care, orthopedic surgery, and neurology were processed and approved without delay. No outstanding authorization issues or claim denials are noted in the record for the encounters contained in this chart export.

NOTE: This document is a synthetic/test medical record generated for demonstration purposes based on a de-identified template. It does not represent a real patient.

ENCOUNTER 1 OF 3

Date of Service: 2024-06-15 | Type: OUTPATIENT VISIT | Specialty: Orthopedic Surgery | Setting: Outpatient | Acuity: LOW

Attending Provider:	Dr. Anita Reyes, MD	Location:	New Amandaville Orthopedic & Sports Medicine Clinic
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Chief Complaint

Numbness and tingling in the left toes, present for approximately 6 months.

History of Present Illness

Ms. Gardner is a 44-year-old female who presents for initial evaluation of numbness and tingling in the left toes with occasional radiation into the left foot. She describes the onset as gradual, beginning roughly six months prior to this visit, with a constant pins-and-needles quality that does not fully resolve throughout the day. She rates the discomfort at 3/10 in severity. Symptoms are aggravated by prolonged standing or walking, which is notable given the standing/walking demands of her occupation, and are partially relieved with rest and elevation of the left leg. She denies any preceding trauma, recent falls, or new footwear. She has not previously sought care for this specific complaint but has self-treated with over-the-counter NSAIDs and acetaminophen with minimal relief. She denies bowel or bladder dysfunction, denies bilateral symptoms, and denies fever or systemic illness. She reports mild intermittent swelling of the left foot and occasional shooting pains radiating up the left leg, but denies redness, warmth, or open wounds. Given the chronicity and functional impact on her occupation, she was referred to orthopedic surgery by her primary care provider for further work-up.

She denies any history of diabetes mellitus, though given her constellation of symptoms a metabolic and structural work-up is indicated to rule out peripheral neuropathy from endocrine, compressive, or structural etiologies. She has not had any prior imaging of the foot or ankle. She is independently ambulatory without assistive devices and reports no falls related to the numbness.

Review of Systems (12-point)

System	Findings
Constitutional	Denies fever, chills, or unintentional weight loss.
HEENT	History of seasonal allergies; denies vision changes or hearing loss.
Cardiovascular	History of hypertension, well controlled; denies chest pain or palpitations.
Respiratory	History of mild intermittent asthma; denies current dyspnea or wheeze.
Gastrointestinal	Reports occasional constipation; denies nausea, vomiting, or GI bleeding.
Genitourinary	Reports occasional urinary frequency; denies dysuria or hematuria.
Musculoskeletal	Reports mild left knee stiffness; denies acute joint swelling elsewhere.
Neurological	Reports numbness/tingling in left toes; denies headache, seizure, or syncope.
Psychiatric	History of generalized anxiety, stable on no pharmacotherapy; denies SI/HI.
Endocrine	History of hypothyroidism, on levothyroxine; denies heat/cold intolerance.
Integumentary	Reports occasional dry skin; denies rash or new lesions.
Hematologic/Lymphatic	History of mild anemia; denies easy bruising or lymphadenopathy.

Past Medical History

Chronic conditions include essential hypertension (dx. 2016), mild intermittent asthma (dx. childhood), and primary hypothyroidism (dx. 2019, on levothyroxine replacement). Surgical history is notable for a low-transverse cesarean section (2010) and a tonsillectomy (childhood). The patient was hospitalized previously for community-acquired pneumonia (2015) and following a motor vehicle collision (2018) with no long-term sequelae reported at that time.

Family History

Mother: hypertension, type 2 diabetes mellitus, living. Father: coronary artery disease, myocardial infarction at age 61, living. One sibling (sister): hypothyroidism. No known family history of malignancy, bleeding disorders, or hereditary neuropathy.

Social History

The patient is a non-smoker with no history of tobacco use and denies use of illicit substances. She reports occasional social alcohol use (1-2 drinks per week) and regular caffeine intake (1-2 cups of coffee daily). She works as an administrative coordinator, a role that requires prolonged standing and walking throughout the day. She is married and lives with her spouse and two children. She reports a generally sedentary exercise pattern with occasional weekend walking.

Current Medications

Medication	Dose	Route	Frequency	Indication
Aspirin	81 mg	PO	Daily	Cardioprotective
Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain

Allergies

No known drug allergies (NKDA). No known food or environmental allergies of clinical significance documented.

Vital Signs

Parameter	Value	Parameter	Value
Blood Pressure	128/72 mmHg	Heart Rate	68 bpm
Temperature	98.2 F	Respiratory Rate	16 breaths/min
Pain Score	3/10	SpO2	97% RA
Weight	156 lbs	Height	5 ft 5 in
BMI	26.0 kg/m2		

Physical Examination

General

Well-developed, well-nourished female in no acute distress, ambulating with a normal, non-antalgic gait.

Cardiovascular

Regular rate and rhythm, S1/S2 normal, no murmurs, gallops, or rubs. No jugular venous distension. 2+ pedal pulses bilaterally.

Respiratory

Clear to auscultation bilaterally, no wheezes, rhonchi, or crackles. Symmetric chest wall expansion.

Musculoskeletal

Left foot with mild dorsal swelling, no erythema or warmth. No palpable mass appreciated on this initial exam. Mild stiffness of the left knee with full, painless range of motion. Full range of motion of the left hip and ankle. Tinel's sign equivocal at the level of the tarsal tunnel.

Neurological

Intact sensation to light touch and pinprick in the right lower extremity. Decreased sensation to light touch and pinprick over the dorsal aspect of the left toes and forefoot. 2+ deep tendon reflexes bilaterally at the patella and Achilles. Motor strength 5/5 throughout bilateral lower extremities. No clonus.

Skin

No rash, ulceration, or skin breakdown noted on either lower extremity.

Diagnostic Studies Ordered

Test	Result	Reference Range	Flag
Complete Blood Count (CBC)	Pending	See reference lab	-
Comprehensive Metabolic Panel (CMP)	Pending	See reference lab	-
Hemoglobin A1c	Pending	< 5.7 %	-
TSH	Pending	0.4 - 4.0 uIU/mL	-
X-ray, Left Foot (3 views)	Pending	N/A	-

Assessment

1. Left forefoot/toe paresthesia and numbness, chronic, of uncertain etiology; differential includes tarsal tunnel syndrome, Morton's neuroma, peripheral neuropathy (metabolic vs. compressive), and early degenerative changes of the left foot.
2. Hypertension, controlled on current regimen.
3. Mild intermittent asthma, currently well controlled.
4. Primary hypothyroidism, on stable levothyroxine dosing.

Plan

1. Obtain CBC, CMP, hemoglobin A1c, and TSH to screen for metabolic causes of peripheral neuropathy.
2. Obtain 3-view weight-bearing X-ray of the left foot to evaluate for structural or osseous abnormality.

3. Trial of a course of formal physical therapy focused on gait mechanics, stretching, and nerve gliding exercises, 2x weekly for 4 weeks.
4. Continue current home medications without change at this time.
5. Recommend supportive, cushioned footwear and activity modification to minimize prolonged standing where feasible.
6. Return to clinic in approximately 8 weeks to review labs, imaging, and response to physical therapy, sooner if symptoms acutely worsen or if she develops weakness, bowel/bladder changes, or bilateral symptoms.

Patient Education / Instructions

The patient was counseled on the benign nature of the working differential at this stage, the rationale for laboratory and imaging work-up, and expected physical therapy course. She was advised to seek urgent care if she develops acute weakness, loss of bladder or bowel control, rapidly progressive symptoms, or signs of infection (fever, spreading redness, warmth) in the affected extremity. She verbalized understanding and agreement with the plan.

Care Team

Name	Role	Contact
Anita Reyes, MD	Attending Orthopedic Surgeon	555-201-4410
Daniel Ho, PA-C	Orthopedic Physician Assistant	555-201-4415
Karen Ibbotson, RN	Clinic Nurse	555-201-4420
Dr. Susan Whitfield, MD	Primary Care Physician (referring)	555-201-3300

Pain & Functional Assessment Questionnaire

Question	Response
Average pain over past week (0-10 scale)	3/10
Worst pain over past week (0-10 scale)	5/10
Does pain interfere with walking?	Mild interference
Does pain interfere with sleep?	No
Does pain interfere with work duties?	Mild interference
Any assistive devices used?	None
Overall satisfaction with current function (0-10)	6/10

Care Coordination Note

Referral to physical therapy submitted and authorized through the patient's Kaiser plan; the patient was provided with a list of in-network physical therapy providers near her home in New Amandaville and elected to schedule with Foothill Physical Therapy. Laboratory orders were transmitted electronically to the reference laboratory, and the patient was instructed to complete blood draws within the next 5-7 days. Radiology order for left foot X-ray was scheduled same-day in the outpatient imaging suite following this visit. The clinic nurse reviewed medication reconciliation with the patient, confirming no changes to her home medication list since her last primary care visit. The patient's primary care physician, Dr. Susan Whitfield, was copied on this consultation note for continuity of care. No durable medical equipment was required at this time. The patient was provided clinic contact information and after-hours triage line information in case of worsening symptoms.

Electronically signed by: **Anita Reyes, MD**
Department: Orthopedic Surgery
Signed: 2024-06-15 4:42 PM

ENCOUNTER 2 OF 3

Date of Service: 2024-12-21 | Type: SURGICAL | Specialty: Orthopedic Surgery | Setting: Inpatient | Acuity: LOW

Attending Provider:	Dr. Anita Reyes, MD	Location:	New Amandaville Regional Medical Center - Main OR
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Chief Complaint

Numbness and tingling in the left toes, now with MRI-confirmed compressive lesion, presenting for surgical excision.

History of Present Illness

The patient is a 45-year-old female who presents with a chief complaint of numbness and tingling in the left toes. The onset of symptoms was gradual, beginning approximately 6 months prior to her initial evaluation earlier this year. The location of the numbness and tingling is primarily in the left toes, but occasionally radiates to the left foot. The duration of symptoms is constant, with the patient experiencing numbness and tingling throughout the day. The character of the numbness and tingling is described as a pins-and-needles sensation. Aggravating factors include prolonged standing or walking, while relieving factors include rest and elevation of the left leg. The severity of symptoms is mild to moderate, with the patient rating the pain as 3/10.

Prior to presentation for surgery, the patient tried over-the-counter pain medications, including ibuprofen and acetaminophen, with minimal relief. The patient also underwent a trial of physical therapy, which provided some relief but did not completely resolve symptoms. Subsequent nerve conduction studies and electromyography demonstrated focal slowing of the posterior tibial nerve without denervation, and MRI of the left foot/ankle obtained thereafter confirmed a small ganglion cyst compressing the posterior tibial nerve at the level of the tarsal tunnel. Given the confirmed structural, compressive etiology and persistent symptoms despite conservative management, the patient elected to proceed with surgical excision. Associated symptoms present include mild swelling in the left foot and occasional shooting pains in the left leg. Associated symptoms absent include fever, chills, and redness.

Review of Systems (12-point)

System	Findings
Constitutional	Denies fever, chills, or unintentional weight loss.
HEENT	History of seasonal allergies; denies vision changes or hearing loss.
Cardiovascular	History of hypertension, well controlled; denies chest pain or palpitations.
Respiratory	History of mild intermittent asthma; denies current dyspnea or wheeze.
Gastrointestinal	Reports occasional constipation; denies nausea, vomiting, or GI bleeding.
Genitourinary	Reports occasional urinary frequency; denies dysuria or hematuria.
Musculoskeletal	Reports mild left knee stiffness; denies acute joint swelling elsewhere.
Neurological	Reports numbness/tingling in left toes; denies headache, seizure, or syncope.
Psychiatric	History of generalized anxiety, stable on no pharmacotherapy; denies SI/HI.
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Integumentary	Reports occasional dry skin; denies rash or new lesions.
Hematologic/Lymphatic	History of mild anemia; denies easy bruising or lymphadenopathy.

Past Medical History

Chronic conditions include essential hypertension (dx. 2016), mild intermittent asthma (dx. childhood), and primary hypothyroidism (dx. 2019, on levothyroxine replacement). Surgical history is notable for a low-transverse cesarean section (2010) and a tonsillectomy (childhood). The patient was hospitalized previously for community-acquired pneumonia (2015) and following a motor vehicle collision (2018) with no long-term sequelae reported at that time.

Family History

Mother: hypertension, type 2 diabetes mellitus, living. Father: coronary artery disease, myocardial infarction at age 61, living. One sibling (sister): hypothyroidism. No known family history of malignancy, bleeding disorders, or hereditary neuropathy.

Social History

The patient is a non-smoker with no history of tobacco use and denies use of illicit substances. She reports occasional social alcohol use (1-2 drinks per week) and regular caffeine intake (1-2 cups of coffee daily). She works as an administrative coordinator, a role that requires prolonged standing and walking throughout the day. She is married and lives with her spouse and two children. She reports a generally sedentary exercise pattern with occasional weekend walking.

Current Medications

Medication	Dose	Route	Frequency	Indication
Aspirin	81 mg	PO	Daily	Cardioprotective
Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain
Pre-op Cefazolin	2 g	IV	Single pre-op dose	Surgical prophylaxis

Allergies

No known drug allergies (NKDA). No known food or environmental allergies of clinical significance documented.

Vital Signs

Parameter	Value	Parameter	Value
Blood Pressure	125/66 mmHg	Heart Rate	63 bpm
Temperature	98.0 F	Respiratory Rate	16 breaths/min
Pain Score	3/10	SpO2	96% RA
Weight	153 lbs	Height	5 ft 5 in
BMI	25.5 kg/m2		

Pre-Operative Physical Examination

General

Well-developed, well-nourished female in no acute distress.

Cardiovascular

Regular rate and rhythm, S1/S2 normal, no murmurs/gallops/rubs, no JVD, 2+ pedal pulses.

Respiratory

Clear to auscultation bilaterally, no wheezes/rhonchi/crackles, symmetric expansion.

Neurological

Intact sensation to light touch and pinprick in the right lower extremity, decreased sensation to light touch and pinprick in the left lower extremity, most pronounced over the dorsal left toes.

Musculoskeletal

Mild stiffness in the left knee, full range of motion in the left hip and ankle.

Skin

No rash or lesions noted.

Radiology Report - MRI Left Foot/Ankle (Final)

Exam Date:	2024-11-14
Ordering Provider:	Marcus Webb, MD
Interpreting Radiologist:	Dr. Felicia Grant, MD
Clinical History:	Chronic left toe numbness/tingling, abnormal NCS of posterior tibial nerve

Technique

Multiplanar, multisequence MRI of the left foot and ankle was performed without and with intravenous gadolinium contrast.

Findings

There is a well-circumscribed, T2-hyperintense, non-enhancing cystic lesion measuring approximately 1.1 x 0.8 x 0.7 cm situated within the tarsal tunnel, intimately associated with the tendon sheath of the flexor digitorum longus and abutting the posterior tibial nerve, which appears mildly displaced and flattened at this level. No abnormal enhancement is seen within the lesion to suggest a solid or malignant component, findings most consistent with a ganglion cyst. The remainder of the posterior tibial nerve course is unremarkable. No plantar fasciitis, Achilles tendinopathy, or significant tenosynovitis is identified. The osseous structures of the foot and ankle are unremarkable, without stress reaction, fracture, or marrow edema. The visualized musculature is unremarkable. No joint effusion of the ankle or subtalar joint.

Impression

1. Approximately 1.1 cm cystic lesion within the tarsal tunnel, consistent with a ganglion cyst, resulting in mild mass effect upon and displacement of the posterior tibial nerve, correlating with the patient's electrodiagnostic findings and clinical symptoms. 2. No other acute osseous or soft tissue abnormality of the left foot/ankle.

Pre-Operative Diagnostic Summary

Test	Result	Reference Range	Flag
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CBC	Within normal limits	See panel	Normal
CMP	Within normal limits	See panel	Normal
Lipid Panel	Within normal limits	See panel	Normal
MRI, Left Foot/Ankle	Ganglion cyst compressing posterior tibial nerve, tarsal tunnel	N/A	Abnormal
Type & Screen	O positive, antibody screen negative	N/A	Normal

Assessment

The patient has a diagnosis of chronic left foot numbness and tingling secondary to compression of the posterior tibial nerve by a ganglion cyst at the tarsal tunnel. The patient also has a history of hypertension, asthma, and hypothyroidism, all of which are currently controlled and pose no contraindication to surgery.

Pre-Operative Plan

1. Proceed with left foot exploration and excision of ganglion cyst under general anesthesia.
2. Administer pre-operative IV cefazolin for surgical site infection prophylaxis.
3. Continue home medications perioperatively per anesthesia protocol; hold as directed morning of surgery.
4. Post-operative pain management, wound care, and follow-up in 7 days.

Operative Report

Pre-operative Diagnosis:	Chronic left foot numbness and tingling secondary to tarsal tunnel ganglion cyst
Post-operative Diagnosis:	Same as pre-operative diagnosis
Procedure Performed:	Left foot exploration with excision of ganglion cyst, posterior tibial nerve decompression
Surgeon:	Anita Reyes, MD
Assistant:	Daniel Ho, PA-C
Anesthesia:	General anesthesia, endotracheal tube, per Dr. Priya Nair, MD (Anesthesiology)
Anesthesia Start / End:	07:42 / 09:05
Positioning:	Supine, with a pillow under the left leg; tourniquet applied to left thigh
EBL:	50 mL
IV Fluids:	800 mL Lactated Ringer's
Specimens:	Ganglion cyst, sent to pathology for evaluation
Complications:	None

Technique

After confirmation of correct patient and surgical site per institutional time-out protocol, the patient was brought to the operating room and placed in the supine position with a pillow under the left leg. General anesthesia was induced without complication. A pneumatic tourniquet was applied to the left thigh. The left lower extremity was prepped and draped in standard sterile fashion. A curvilinear incision was made over the medial aspect of the ankle, centered over the tarsal tunnel. Dissection was carried down through subcutaneous tissue with careful hemostasis using electrocautery. The flexor retinaculum was identified and incised longitudinally to expose the contents of the tarsal tunnel. Dissection continued down to the level of the posterior tibial nerve, which was found to be compressed by a small ganglion cyst arising adjacent to the flexor digitorum longus tendon sheath. A nerve stimulator was used intermittently to confirm nerve integrity throughout the dissection. The ganglion cyst was carefully dissected free from the surrounding neurovascular structures using a #15 scalpel and Metzenbaum scissors and was excised in its entirety without violation of the nerve epineurium. The posterior tibial nerve was inspected and found to be intact and well-perfused following decompression. Hemostasis was confirmed after tourniquet deflation. The flexor retinaculum was re-approximated loosely with 4-0 Vicryl suture to avoid re-tethering the nerve. The subcutaneous tissue was closed in layers with 4-0 Vicryl suture, and the skin was closed with 4-0 Vicryl in a subcuticular fashion. A sterile dressing was applied, followed by a soft bulky compressive wrap.

Instruments/Implants Used

#15 scalpel, Metzenbaum scissors, nerve stimulator, bipolar electrocautery, 4-0 Vicryl suture.

Disposition

The patient tolerated the procedure well and was transferred to the post-anesthesia care unit (PACU) in stable condition, awake, extubated, and hemodynamically stable.

Post-Anesthesia Care Unit (PACU) Note

The patient arrived to PACU at 09:12 awake, alert, and hemodynamically stable. Initial PACU vitals: BP 118/68 mmHg, HR 72 bpm, RR 14/min, SpO2 99% on 2L nasal cannula, pain 2/10 at the surgical site. Left foot dressing clean, dry, and intact with no evidence of active bleeding. Capillary refill less than 2 seconds at the toes, toes warm and well-perfused, sensation grossly intact to light touch distal to the surgical site (improved compared to pre-operative baseline for the mid-foot, with expected surgical-site numbness at the incision itself). The patient was monitored for 90 minutes per protocol with stable vitals throughout and was discharged from PACU to the inpatient orthopedic floor for overnight

observation given planned inpatient admission status.

Discharge / DME Plan

The patient will be discharged home with a follow-up appointment in 7 days. The patient will be given a prescription for pain management and will be instructed to elevate the left leg and avoid heavy lifting or bending. She will be provided a post-operative surgical shoe and instructed on dressing care and signs of infection to monitor for at home.

Follow-up needed: Yes, in 7 days.

Surgical / Inpatient Care Team

Name	Role	Contact
Anita Reyes, MD	Attending Orthopedic Surgeon	555-201-4410
Daniel Ho, PA-C	First Assist	555-201-4415
Priya Nair, MD	Attending Anesthesiologist	555-201-8810
Wendy Ostrander, CRNA	Anesthesia (nursing)	555-201-8815
Marissa Chu, RN	Circulating Nurse	555-201-8820
Todd Farrow, CST	Scrub Technician	555-201-8825
Lauren Best, RN	PACU Nurse	555-201-8830
Nina Osei, RN	Inpatient Floor Nurse (overnight)	555-201-8840

Inpatient Nursing Flowsheet - Overnight Observation

Time	BP	HR	RR	Temp	SpO2	Pain
09:12	118/68	72	14	97.5F	99% 2L	2/10
11:00	116/70	70	14	97.8F	98% RA	2/10
15:00	120/72	68	15	98.0F	97% RA	3/10
19:00	119/70	66	15	98.1F	97% RA	2/10
23:00	121/74	68	16	98.0F	97% RA	2/10
03:00	117/69	64	15	97.7F	98% RA	1/10
06:30	122/71	66	16	97.9F	98% RA	2/10

The patient remained hemodynamically stable throughout the overnight observation period with well-controlled pain on the standing acetaminophen and PRN oxycodone-acetaminophen regimen. Left foot dressing remained clean, dry, and intact on all checks. Neurovascular checks of the left foot were performed hourly for the first four hours and then every four hours thereafter, with toes consistently warm, well-perfused, and with brisk capillary refill throughout. The patient ambulated to the bathroom with assistance twice overnight without difficulty and tolerated a regular diet. She was cleared for discharge by the surgical team on morning rounds.

Anesthesia Record Summary

ASA Classification:	ASA II
Airway:	Endotracheal tube, Grade I view, atraumatic intubation
Anesthetic Agents:	Propofol induction, sevoflurane maintenance, fentanyl, ondansetron
Tourniquet Time:	42 minutes, left thigh
Fluids:	800 mL Lactated Ringer's
Estimated Blood Loss:	50 mL
Post-op Nausea/Vomiting:	None

Electronically signed by: **Anita Reyes, MD**

Department: Orthopedic Surgery

Signed: 2024-12-21 11:20 AM

ENCOUNTER 3 OF 3

Date of Service: 2025-03-15 | Type: OUTPATIENT VISIT | Specialty: Neurology | Setting: Outpatient | Acuity: LOW

Attending Provider:	Dr. Marcus Webb, MD	Location:	New Amandaville Neurology Associates
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Chief Complaint

Follow-up evaluation approximately 12 weeks status-post left tarsal tunnel decompression, for reassessment of residual sensory symptoms.

Interval History

Ms. Gardner returns to neurology clinic approximately 12 weeks following surgical excision of a ganglion cyst with decompression of the left posterior tibial nerve. She reports substantial ongoing improvement in her symptoms since the last visit. The pins-and-needles sensation in her left toes has almost completely resolved, and she now rates any residual intermittent tingling at 1/10, occurring only rarely after prolonged periods of standing at work, which is a marked improvement from her pre-operative baseline of constant 3/10 symptoms. She denies any residual numbness that interferes with her daily activities, denies weakness, and denies falls. She has completed her post-operative physical therapy course and has returned to her full pre-operative work duties, including prolonged standing, without significant limitation.

She denies any new neurologic complaints, denies symptoms in the contralateral extremity, and denies bowel or bladder dysfunction. She continues to take her home medications for hypertension, asthma, and hypothyroidism without any recent changes or adverse effects.

Review of Systems (12-point)

System	Findings
Constitutional	Denies fever, chills, or unintentional weight loss.
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Past Medical History

Chronic conditions include essential hypertension (dx. 2016), mild intermittent asthma (dx. childhood), and primary hypothyroidism (dx. 2019, on levothyroxine replacement). Surgical history is notable for a low-transverse cesarean section (2010) and a tonsillectomy (childhood). The patient was hospitalized previously for community-acquired

pneumonia (2015) and following a motor vehicle collision (2018) with no long-term sequelae reported at that time.

Family History

Mother: hypertension, type 2 diabetes mellitus, living. Father: coronary artery disease, myocardial infarction at age 61, living. One sibling (sister): hypothyroidism. No known family history of malignancy, bleeding disorders, or hereditary neuropathy.

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The patient is a non-smoker with no history of tobacco use and denies use of illicit substances. She reports occasional social alcohol use (1-2 drinks per week) and regular caffeine intake (1-2 cups of coffee daily). She works as an administrative coordinator, a role that requires prolonged standing and walking throughout the day. She is married and lives with her spouse and two children. She reports a generally sedentary exercise pattern with occasional weekend walking.

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Lisinopril	10 mg	PO	Daily	Hypertension
Albuterol HFA	2.5 mg (nebulized equiv.)	Inhaled	PRN	Asthma
Levothyroxine	50 mcg	PO	Daily	Hypothyroidism
Acetaminophen	1000 mg	PO	PRN	Pain

Allergies

No known drug allergies (NKDA). No known food or environmental allergies of clinical significance documented.

Vital Signs

Parameter	Value	Parameter	Value
Blood Pressure	120/70 mmHg	Heart Rate	64 bpm
Temperature	98.1 F	Respiratory Rate	15 breaths/min
Pain Score	0-1/10	SpO2	99% RA
Weight	152 lbs	Height	5 ft 5 in
BMI	25.3 kg/m2		

Physical Examination

General

Well-appearing female in no acute distress, ambulating normally without assistive devices.

Wound/Incision

The medial ankle incision has healed well with a thin, mature, non-tender scar. No residual swelling, erythema, or induration.

Neurological Examination

Cranial nerves II-XII grossly intact. Motor strength 5/5 throughout bilateral lower extremities including tibialis anterior, gastrocnemius, and extensor hallucis longus. Sensation to light touch and pinprick now grossly intact and symmetric throughout the left foot and toes, a marked improvement compared to prior exams, with only trace subjective diminished sensation immediately adjacent to the surgical scar reported by the patient but not clearly reproducible on formal testing today. Tinel's sign at the tarsal tunnel is now negative. Deep tendon reflexes 2+ and symmetric. Gait is normal-based, non-antalgic; heel walk and toe walk performed without difficulty. Romberg negative.

Repeat Diagnostic Studies

Test	Result	Reference Range	Flag
Repeat Nerve Conduction Study, left posterior tibial nerve	Near-normalized conduction velocity	Normal conduction velocity	Improved / Near Normal

Repeat nerve conduction study of the left posterior tibial nerve demonstrates near-normalized conduction velocity across the tarsal tunnel, consistent with the patient's reported clinical improvement and confirming successful decompression.

Assessment

1. Resolved/resolving left tarsal tunnel neuropathy, status post surgical decompression, with excellent clinical and electrodiagnostic improvement. No evidence of recurrence.
2. Hypertension, controlled.
3. Mild intermittent asthma, controlled.
4. Primary hypothyroidism, controlled.

Plan

1. No further physical therapy required at this time; encourage continued independent home stretching program as needed.
2. No further neurology follow-up scheduled at this time; the patient may be discharged from active neurology follow-up.
3. Continue routine primary care follow-up for management of hypertension, asthma, and hypothyroidism.
4. Return to neurology clinic as needed if symptoms recur or new neurologic symptoms develop.
5. Continue home medications without change.

Patient Education / Instructions

The patient was informed of the excellent clinical and electrodiagnostic recovery and the low likelihood of recurrence. She was advised to contact the clinic should any numbness, tingling, or weakness recur or worsen in the future. She expressed satisfaction with her surgical outcome and had no further questions at today's visit. This concludes the active treatment course for this condition.

Care Team

Name	Role	Contact
Marcus Webb, MD	Attending Neurologist	555-201-7710
Priya Chandrasekaran, RN	Neurology Clinic Nurse	555-201-7715
Dr. Anita Reyes, MD	Orthopedic Surgery (co-managing, visit concluded)	555-201-4410
Dr. Susan Whitfield, MD	Primary Care Physician	555-201-3300

Pain & Functional Assessment Questionnaire

Question	Response
Average residual symptom severity over past week (0-10)	1/10
Frequency of residual tingling	Rare, only after prolonged standing
Interference with work duties	None
Interference with sleep	None
Return to baseline activity level	Yes, fully returned
Overall satisfaction with treatment outcome (0-10)	9/10

Symptom Trend Summary Across Encounters

Time	BP	HR	RR	Temp	SpO2	Pain
2024-06-15	128/72	68	16	98.2F	97% RA	3/10
2024-08-10	124/70	70	16	97.9F	98% RA	3/10
2024-12-21	125/66	63	16	98.0F	96% RA	3/10
2024-12-28	122/68	66	16	97.8F	98% RA	2/10
2025-03-15	120/70	64	15	98.1F	99% RA	0-1/10

The table above summarizes vital signs and reported pain/symptom severity across all five encounters in this chart, demonstrating a stable cardiopulmonary status throughout the treatment course and a clear downward trend in reported symptom severity from a baseline of 3/10 constant symptoms to near-complete resolution (0-1/10, intermittent) following surgical decompression and post-operative recovery.

Long-Term Follow-Up Recommendations

Given the excellent surgical and electrodiagnostic outcome, no routine long-term neurology surveillance is recommended for this condition. The patient was counseled that a small minority of tarsal tunnel decompression patients may experience recurrence of symptoms years later, most commonly due to scar tissue formation or, rarely, cyst recurrence, and that such recurrence would warrant repeat clinical evaluation and, if indicated, repeat electrodiagnostic testing and imaging. She was also encouraged to maintain supportive footwear during prolonged standing at work as a general preventive measure, and to continue the independent home stretching and nerve-gliding exercises she learned during physical therapy on an as-needed basis.

Care Coordination Note / Chart Closure

This visit concludes the active neurology and orthopedic surgery treatment course for the patient's left tarsal tunnel neuropathy. A summary letter was sent to the patient's primary care physician, Dr. Susan Whitfield, documenting the full course of treatment from initial presentation through surgical resolution, for inclusion in the primary care longitudinal record. The patient was advised to continue routine primary care follow-up for her chronic conditions (hypertension, asthma, hypothyroidism) and was provided standard instructions to re-contact either specialty clinic directly should any related symptoms recur in the future. No further specialty appointments are scheduled at this time.

Electronically signed by: **Marcus Webb, MD**

Department: Neurology

Signed: 2025-03-15 3:30 PM